THE MATERNITY METHOD

NCLEX CHEAT SHEET

Hyperbilirubinemia & Hemolytic Disease of the Newborn

Maternal & Newborn › Newborn Complications

The Big Picture

Newborns break down lots of red blood cells, and their young livers clear **bilirubin** slowly, so many get **jaundice**. Most is normal (**physiologic**). Jaundice that comes **too early**, rises **too fast**, or lasts **too long** is **pathologic**. Too much unconjugated bilirubin can reach the brain (**kernicterus**). **Hemolytic disease** happens when mom's antibodies destroy baby's red cells (Rh or ABO).

KNOW THE NUMBERS

< 24 hrs	Day 3–5	28 wks	72 hrs	8–12×
Jaundice this early = **always abnormal**	Normal jaundice **peaks** (term)	**RhoGAM** for Rh− mom	RhoGAM **after birth** (Rh+ baby)	**Feeds a day** flush bilirubin

Types of Jaundice

Type	When / why	What to do
Physiologic	Appears **after 24 hrs**, peaks day 3–5, fades in 1–2 wks. Immature liver	Frequent feeds, monitor levels
Pathologic	**Within 24 hrs**, rises fast, or lasts > 2 wks. Hemolysis (Rh/ABO, G6PD), infection, bruising/cephalohematoma, prematurity	Find the cause; **phototherapy**; possibly exchange transfusion
Breastfeeding jaundice	**First week**: baby isn't getting **enough milk** → fewer stools	**Feed more often** (8–12×/day), lactation help
Breast milk jaundice	**After the first week**: something in the milk; baby is otherwise well	Usually **keep breastfeeding**; monitor

Hemolytic Disease

Rh Incompatibility

- **Rh– mom + Rh+ baby**
- Mom gets exposed to baby's blood → makes **anti-D antibodies** (sensitized)
- **Next** Rh+ baby: antibodies cross the placenta and destroy RBCs → anemia, jaundice, **hydrops fetalis**
- Usually **not** a problem in the first pregnancy

ABO Incompatibility

- **Mom type O + baby type A or B**
- Can happen in the **first** pregnancy
- Usually **milder** than Rh
- Watch for **early** jaundice

RhoGAM (Rh Immune Globulin)

- **Who:** **Rh– mom who is NOT sensitized** (negative indirect Coombs)
- **When:** about **28 weeks** + within **72 hrs** after birth **if baby is Rh+**
- **Also after:** miscarriage, ectopic, abortion, amniocentesis, bleeding, belly trauma, ECV
- **IM injection**; it's a **blood product** (consent, check ID); prevents mom from making antibodies
- **Cord blood:** baby's type, Rh, and **direct Coombs** (looks for antibodies on baby's RBCs)

Spot It

- Yellow skin that spreads **head → toes**
- **Press** on nose or forehead: yellow when blanched
- Yellow **sclera** and gums
- Measured with **transcutaneous** or **serum** bilirubin, plotted by **hours of age**

Kernicterus Warning Signs

- **Lethargy**, poor suck
- **High-pitched cry**
- Floppy tone → then **stiff, arching** back
- **Seizures**
- Can cause **permanent** brain damage and hearing loss

MEMORY HACK · PHOTOTHERAPY CARE

Under the lights: 5 S's

Shield	Skin	Switch	Supply	Survey
eyes (remove for feeds)	**bare**: diaper only, no lotions	position **every 2 hrs**	**extra feeds**: fluid loss	**temp**, I&O, daily weight, skin

Expected with phototherapy: loose **green stools**, temporary **rash**, more fluid loss. **Turn lights off** when drawing a bilirubin level. Home **biliblanket** may be used for mild cases.

☆ MUST-KNOW FOR THE NCLEX

- **Jaundice in the first 24 hours = pathologic.** Report it.
- **RhoGAM** goes to the **Rh– mom**, never the baby. Give at 28 wks and within 72 hrs after an **Rh+** birth.
- **Phototherapy:** cover eyes, expose skin, turn q2h, increase feeds, no lotions.
- Breastfeeding jaundice = **not enough milk**, so **feed more**.

QUICK CHECK · TEST YOURSELF

1. Jaundice appearing at 18 hours of age is _____.
2. RhoGAM is given within ___ hours after birth if the baby is Rh+.
3. During phototherapy, the baby's _____ must be covered.
4. ABO incompatibility usually involves a mom with blood type ___.
5. A high-pitched cry and arching in a jaundiced baby suggest _____.

Answers: 1. pathologic · 2. 72 · 3. eyes · 4. O · 5. kernicterus (bilirubin encephalopathy)

For study use only. Always follow facility protocols, current guidelines, and provider orders.